

CH SHARIF AND SAEED HOSPITAL

Complete Workflow Architecture v7.2

Cross-portal workflow from patient intake to clinical discharge, central billing, department settlement and doctor finance.

FINAL SOURCE OF TRUTH - VERSION 7.2

Admission begins at Front Desk. Department bills remain separate. Central collections are allocated department-wise. Doctor discharge re-authentication, outsourced department settlement, Panel receivables, high-cost medicine approval, doctor-sponsored discount and doctor tax rules are included.

Supersedes v7.1 for all revised admission, billing, department settlement, doctor finance and panel-flow rules.

1. Self-Pay Admission End-to-End

Self-Pay admission

1	Front Desk creates temporary admission identity and admission file.
2	Optional advance is collected and stored as patient credit.
3	Admission Portal manages stay, services and department requests.
4	Charges accumulate in separate Hospital/Lab/Pharmacy/Neurology/other department invoices.
5	Partial payments may be made at Front Desk at any time with explicit department allocation.
6	Interim statement may be printed at any time.
7	Doctor re-authenticates and clinically discharges patient with summary.
8	Front Desk performs final billing, applies prior credits/payments and collects remaining patient due.
9	Department/provider payables remain available for later authorized settlement.

2. Panel Admission End-to-End

Panel admission

1	Front Desk searches permanent Panel Patient Registry and validates active membership/panel.
2	Resolve department/service tariff, coverage, co-pay, exclusions and any required panel authorization reference.
3	Create admission and optionally collect patient advance/co-pay if policy requires.
4	Admission adds department services and separate sub-invoices calculate Patient Share plus Panel Receivable.
5	Interim statement shows patient responsibility separately from panel responsibility.
6	Doctor clinically discharges after credential re-authentication.
7	Front Desk clears patient share and finalizes panel claim/receivable references.
8	Panel remittance later realizes the panel receivable by department.
9	Outsourced provider settlement uses realized amounts only: collected patient share plus panel cash actually received.

3. High-Cost Medicine Gate

High-cost medicine

1	Admission/doctor requests medicine under Hospital Managed mode.
2	System calculates line amount = quantity x approved rate and compares with configured threshold.
3	If threshold exceeded, request status becomes Authorization Required.
4	Attendant confirmation and/or Admin/Super Admin credential approval is captured per policy.
5	Only Authorized request proceeds to Pharmacy. Rejected/Pending cannot be dispensed.

4. Outsourced Department Settlement

Settlement

1	Department invoice is billed to patient/panel and Front Desk collects/records realizations.
2	Provider ledger accumulates realized payable amount according to provider contract.
3	Authorized Billing/Admin/Super Admin opens settlement.
4	Full or Partial amount is paid by Cash/Bank/Cheque/Online according to provider setup.
5	Provider representative acknowledgement is captured and voucher printed.
6	Remaining payable stays outstanding.

5. Doctor Financial Flow

Doctor financial flow

1	Eligible completed service creates doctor commission accrual using configured Fixed/% rule and Gross/Net commission base.
2	Doctor-sponsored patient discount is deducted from that doctor's commission payable.
3	Commission tax is calculated using doctor commission tax profile effective for the period.
4	Salary is generated separately from attendance and salary profile; salary tax applies separately.
5	Salary and Commission remain independent payment/outstanding streams.